where critical gaps exist. It offers only limited collaboration with private-sector healthcare providers, ‘especially those working in rural and remote areas or with under-served communities’ who could be ‘offered encouragement through provision of appropriate skills to meet public health goals’.

The policy only deals with charitable private-sector providers. It further notes that it supports ‘voluntary service in rural and under-served areas on a pro-bono basis by recognized healthcare professionals under a “giving back to society” initiative’. Another form of collaboration sought to be encouraged is ‘engagement in public goods, where the private sector contributes to preventive or promotive services without profit—as part of CSR work or on contractual terms with the government’.

It seeks to use strategic purchasing as stewardship to direct investment for the commercial health sector. Critical gaps in public health services would be filled with strategic purchasing, which would encourage the private sector to invest in capacity creation to provide these services. The main mechanism of strategic purchasing would be insurance and through trusts. The government also proposed to buy in the private sector in urban primary healthcare by building partnerships to address specific gaps like diagnostic, ambulance, safe blood, rehabilitative and palliative services, mental healthcare and telemedicine.